

Bed Bug Bite - Female (Child)

Algorithm 1180 | Child (0-17 years) | Gender at birth: Female

Definition

- Qatar-localized UAT-only adult and pediatric pathway for suspected bed-bug bites, screening for anaphylaxis, infection, extensive reactions, immune risk and environmental/safeguarding concerns; not approved for production.
- Child patient.
- Female.

Initial assessment

1. [LOCATION] Where are the bites?
2. [DURATION] When did the bites appear?
3. [YES_NO] Any swelling of the lips, throat, or difficulty breathing?
4. [OPEN_TEXT] What is the exact age; is the patient pregnant; are bites widespread or near an eye/mouth; and are there fever, severe pain, spreading redness, immune suppression, infestation affecting a vulnerable person, neglect or safeguarding concerns?
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Anaphylaxis

Are throat or tongue swelling, breathing difficulty, faintness, confusion, or collapse the main concern?

Systemic allergic-reaction features require the Anaphylaxis pathway.

1. Disposition level 100

Are the lips, mouth, throat, or tongue suddenly swollen, is the person struggling to breathe, or has anyone lost consciousness or become floppy?

NHS.UK insect bites and stings guidance lists these as signs of a serious allergic reaction (anaphylaxis) requiring an immediate 999 call.

1. Disposition level 50

Is the skin around the bites hot, red, and painful, or is pus or fluid coming out (signs of infection)?

NHS.UK guidance flags these infection signs as needing pharmacist advice or a primary-care review.

1. Disposition level 50

After emergency and infection features are excluded, are these presumed bed-bug bites requiring confirmation and safe symptom/environmental advice?

The cause cannot be confirmed remotely, and pediatric, pregnancy, medication and vulnerable-household risks require clinician review in this UAT pathway.

Advice

Emergency allergic reaction precautions

- Call Qatar 999 immediately and follow dispatcher instructions. If the patient has their own prescribed adrenaline auto-injector, use it according to its plan. Keep the patient lying down unless breathing is difficult; do not let them stand or walk and do not drive. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: symptoms persist or recur; loss of consciousness.

Routine bite follow-up

- Keep the area clean and book a routine review for signs of infection.
- Call Back If: increasing redness or pain; fever develops.

Clinical and environmental review for presumed bites

- Avoid scratching, gently clean affected skin, and arrange clinician or pharmacist review before medication is selected. Use a licensed pest-control process appropriate to the household; do not apply pesticides to skin or improvise chemical treatments.
- Call Back If: pain, spreading redness, discharge or fever; allergic reaction symptoms.

First aid

- Call Qatar 999 immediately and follow dispatcher instructions. If the patient has their own prescribed adrenaline auto-injector, use it according to its plan. Keep the patient lying down unless breathing is difficult; do not let them stand or walk and do not drive. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Insect bites and stings", <https://www.nhs.uk/conditions/insect-bites-and-stings/> (page last reviewed 01 June 2023)

Provenance

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redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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